LIBBY — PATIENT/CAREGIVER TRACK

demo-nsclc-egfr-l858r-post-osi-d3m0

Plain-language summary for patient and caregiver

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This is decision-support information, not a treatment plan. Please share it with your oncologist before making any decisions based on what's here.

What this page is

Libby is an experimental tool. It pulls together published research on cancer treatments that might fit a particular case, then runs the options past five "reviewers" who each weight things differently. One reviewer is a risk-taker. One is conservative about side effects. One is the published-evidence skeptic. One is a patient advocate. One cares mainly about whether the regimen lines up with official guidelines. They argue. Sometimes they disagree on purpose.

This page is the patient-facing version of what they came up with. The case is fictional, used to demo the tool, but the drugs and trial data are real.

What we know about the cancer

This is lung cancer (the adenocarcinoma type) that has spread beyond the lung. Stage IV. Two specific genetic features are driving it, and both have drugs aimed at them:

- A change in a gene called EGFR, specifically the L858R mutation.
- Extra copies of a gene called MET. Doctors call this MET amplification.

The patient was on a pill called osimertinib first. It targets the EGFR change, and it worked for about 22 months before the cancer started growing again. The MET amplification almost certainly explains why osimertinib stopped working. That matters because it points pretty directly at what should come next.

What the patient said matters most

A pill, ideally. Not hours of infusions in a chair. The patient is a working artist, so anything that risks numbness or weakness in the hands is more than a quality-of-life concern, it is a livelihood concern. So the things she said she wanted to avoid were:

- Severe nerve damage.
- Heart problems.
- Hair loss.

She is open to a clinical trial, which expands what the board could put on the table. Asked whether to push for the most effective treatment even if it is rougher, or the gentlest treatment that still works, she landed slightly on the effectiveness side. About 55/45. Not a strong lean.

The options the board considered

Option 1 — savolitinib + osimertinib (preferably as part of a clinical trial)

The board recommended this. Four of five reviewers endorsed it; the fifth agreed with caveats.

Two pills, both taken at home. Osimertinib is the same drug she was already on. Savolitinib gets added on top, and it blocks the MET pathway, which is the same pathway that probably caused the resistance in the first place.

So the drug is aimed at the thing that broke. There is earlier-phase data behind it (a trial called TATTON), and a larger phase-3 trial (called SAFFRON) is recruiting now.

What the upside might look like, in plain numbers: in TATTON, about 1 in 3 patients in this exact biomarker setup had measurable tumor shrinkage. The published numbers suggest this combination can put the disease back under control for a meaningful share of patients, but I cannot give you a precise long-term improvement here without more context. The phase-3 trial is what will eventually quantify how much extra time it buys.

Side effects to watch for: liver-test changes (caught with routine blood draws), some swelling in the legs, fatigue. None of these match her list of vetoes.

Does it match what she wanted? Yes on every axis. All-oral, no infusions, no severe neuropathy or heart issues or hair loss in the published reports, and there is a trial route. The fifth reviewer, the guideline reviewer, pointed out that this combination is not yet officially listed in the major guidelines as a recommended regimen, so the cleanest path is via the trial. Which she had already said she was open to.

Option 2 — amivantamab + lazertinib *(considered with caveats)*

The board considered this seriously and did not recommend it as the first choice.

It is a combination of two drugs. One of them, amivantamab, is given by IV infusion in the clinic, especially at the start. The other, lazertinib, is a pill at home. Amivantamab is an antibody that hits both EGFR and MET, which is why it shows up here.

The efficacy signal is real. In a study called CHRYSALIS-2, about 36 of every 100 patients in a similar situation had measurable tumor shrinkage. That is slightly higher than Option 1.

The problem is the side-effect side. Infusion reactions are common and sometimes severe. Rash is nearly universal. And the combination data show meaningful nerve-related side effects. The conservative reviewer issued a veto on exactly that ground: the neuropathy risk for a working artist plus the IV burden. Both directly conflict with what she said she wanted. The advocate reviewer also dissented on preference fit. The skeptic reviewer dissented on evidence quality, because CHRYSALIS-2 is single-arm without a randomized comparator in the post-osimertinib subset.

It is still on this page because two of the five reviewers (the risk-taker, on effect size; the guideline reviewer, on NCCN status) did endorse it, and Libby's rule is that a vetoed option does not silently disappear. The patient and the oncologist should be able to see what was put on the table and rejected. But the board's bottom line is: this is a real option only if Option 1 fails or proves intolerable. The slightly larger response rate over Option 1 was not judged enough to override the stated preferences.

Option 3 — patritumab deruxtecan (HER3-DXd) *(considered with caveats)*

The board did not recommend this.

It is an antibody-drug conjugate, given by IV every three weeks. It works by carrying a chemotherapy payload to cells that display a marker called HER3.

Hair loss is a frequent side effect of this drug. She said upfront that hair loss was something she wanted to avoid, and the advocate reviewer took her at her word and removed the option from active consideration. There is also a small but real risk of lung inflammation (the medical term is ILD, interstitial lung disease) that anyone considering this drug needs to know about. Only one reviewer, the risk-taker, kept it in the running, on effect-size grounds.

Why it is still on this page: transparency. If she ever wants to reopen the conversation about the hair-loss veto, this is a real third-line option worth a discussion. But the dossier surfaces it as a what-if, not a recommendation.

Questions to ask your oncologist

1. Am I a candidate for the SAFFRON or SACHI trial of savolitinib plus osimertinib? Where is the nearest enrolling site, and what is the expected timeline to a slot?
2. If a trial slot is not available, can savolitinib plus osimertinib be accessed off-trial? What does that path look like, and what are the trade-offs?
3. With Option 1, what is the monitoring plan for liver changes? What numbers would push us to pause or stop the drug, and what would we do then?
4. If Option 1 stops working down the line, what is the realistic next step? Is Option 2 back on the table at that point, with the neuropathy risk discussed openly, or would we look at chemotherapy first?
5. Has the guideline status of any of these options shifted in the last six months? Anything I should know about that the page might not reflect?
6. What is the realistic time-to-response for Option 1, and what scans or markers will we use to know it is working?
7. Given my work with my hands, are there specific side effects I should be planning around even on Option 1, where the board did not flag any vetoes?
8. If I ever wanted to reopen the hair-loss question and look at Option 3, what would change in the plan, and what would the conversation look like?

Where to read more

- Clinician summary— the technical version of this page.
- Trial table— the trials Libby reviewed.
- Evidence list— the clinical and lab studies cited.
- Tumor-board transcript— what each reviewer actually said.
- Recommendations detail— full structured table.

Sources

The recommendations cited the following PubMed records and clinical-trial registrations:

- PMID 32679432 (TATTON expansion, savolitinib + osimertinib)
- PMID 36720074 (CHRYSALIS-2, amivantamab + lazertinib)
- PMID 37563559 (HERTHENA-Lung01, patritumab deruxtecan)
- NCT05261399 (SAFFRON phase-3 trial registry)